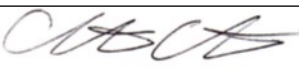


Guideline Title: Pulmonary Embolism

Guideline Number: GUID-3203-M014

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** To recognize the patient with pulmonary embolism (PE) and to provide and maintain adequate oxygenation, ventilation, and tissue perfusion. To provide emergent treatment and transport.
- II. **DEFINITIONS:**
- III. **DEPARTMENT GUIDELINE:**
 - a. Establish care as defined by GUID3203-G002 – *General Patient Care*.
 - b. Assess patient for:
 - i. Massive/submassive PE: hypotension, poor perfusion to the extremities, tachycardia, tachypnea, hypoxia, signs of right heart strain (right axis EKG, bedside sonogram/CT findings, elevated troponin).
 - ii. Acute pulmonary infarction: localized tenderness, signs of pleural effusion, and diminished breath sounds
 - iii. Embolism without infarction: tachypnea, tachycardia, pleuritic pain, crackles, or wheeze
 - iv. Multiple pulmonary emboli or thrombi: pulmonary hypertension and cor pulmonale
 - a. Treat hypoxia with supplemental oxygen (goal SPO2 > 90%)
 - i. If severe respiratory distress treat per GUID3203-G008 - *Rapid Sequence Intubation*
 - b. Treat hypotension per GUID3203-M011 - *Hypotension*.
 - c. Consider anticoagulation therapy. Obtain orders and medication from sending/receiving physician if no contraindications. **Heparin IV bolus 80 units/kg** (Max 5000 units) followed by **Heparin infusion at 18 units/kg/hr** (Max 1300 units/hr). Do not start heparin if the patient has received Lovenox.
 - d. For persistent hypotension despite management with the preceding measures, consider thrombolytic therapy (per sending physician) prior to departing the referring facility.
 - e. Treat pain and/or anxiety per GUID3203-G005 - *Analgesia and Sedation Management*
- IV. **DOCUMENTATION:** EMS Charts
- V. **REFERENCES:**